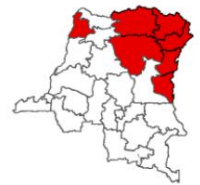




National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°128/MVEBDB/09/19/2026

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HIGHLIGHTS (last 24 hours)



Over the past 24 hours, 58 new confirmed cases of Ebola Virus Disease (EVD), including 13 community deaths, have been reported in four provinces: Ituri (38 cases), North Kivu (14 cases), Haut-Uélé (5 cases), and Tshopo (1 case). In addition, 10 deaths in Ebola Treatment Centers (ETCs) have been recorded.

📌 No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 7,672	ACCUMULATIVE DEATH CONFIRMED 3,699	LETHALITY 48.2%	PATIENTS IN ISOLATION/CTE 886	ACCUMULATIVE HEALED 1879	TRACKING RATES CONTACTS (Of the Day) 79.9%
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Affected Provinces and Affected Health Zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo, Bas-Uélé and South Ubangi



63 out of 167 health zones affected (37.7%)

Summary of key points from the last 24 hours

As of September 19, 2026, a total of **58 new confirmed cases** of Ebola Virus Disease (EVD) were reported, including **13 community deaths**. These confirmed cases originated from the provinces of **Ituri (38 cases), North Kivu (14 cases), Haut-Uélé (5 cases) and Tshopo (1 case)**.

However, **10 intra-CTE deaths** (6 in Ituri, 3 in North Kivu and 1 in Tshopo) were recorded, bringing the cumulative number of deaths for the day to **23**.

Furthermore, **15 patients** have been declared recovered (**13 in Ituri and 2 in North Kivu**) after obtaining two negative results for the MVE.

Since the start of the epidemic in the Democratic Republic of Congo (DRC), a cumulative total of **7,672 confirmed cases, including 3,699 deaths**, has been recorded, representing a case fatality rate of **48.2 %**. The epicenter remains the Ituri province, which accounts for **77.1% of the cases**. confirmed cases reported in the 7 affected provinces and also represents **(65.5 %)** of new confirmed cases.

No new health zones have been affected in the last 24 hours.

Thus, **63 out of 167 health zones (37.7%)** are affected in **7 provinces**. Ituri has 28 out of 36 health zones affected, followed by North Kivu (16/34), Haut-Uélé (7/13), Tshopo (7/23), South Kivu (1/34), Bas-Uélé (3/11) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 19, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	38	5913	2,703	45.7%	28/36 (77.8%)
North Kivu	14	1402	850	60.6%	16/34 (47.1%)
Haut-Uélé	5	307	126	41.0%	7/13 (53.8%)
Tshopo	1	39	14	35.9%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	7	4	57.1%	3/11 (27.3%)
South Ubangi	0	1	1	100.0%	1/16 (6.3%)
Total	58	7,672	3,699	48.2%	63/167 (37.7%)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 19, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5913	2703	45.7%	38	9	6	15
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	193	39	20.2%	6	0		0
Boga	2	2	100.0%				0
Bunia	1654	517	31.3%	8	4		4
Damascus	40	9	22.5%				0
Drodro	15	7	46.7%				0
Fataki	81	30	37.0%	1	0		0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	34	12	35.3%				0
Komanda	147	89	60.5%	1	0		0
Lita	273	135	49.5%	1	0		0
Logo	13	5	38.5%				0
Lolwa	31	8	25.8%				0
Mahagi	4	1	25.0%				0
Mambasa	21	9	42.9%				0
Mandima	59	33	55.9%				0
Mangala	333	146	43.8%	2	0		0
Mongbwalu	686	319	46.5%	7	4		4
Nia-Nia	266	141	53.0%	4	1		1
Nizi	754	274	36.3%	5	0		0
Nyankunde	126	36	28.6%				0
Rimba	12	4	33.3%				0
Rwampara	1046	352	33.7%	3	0		0

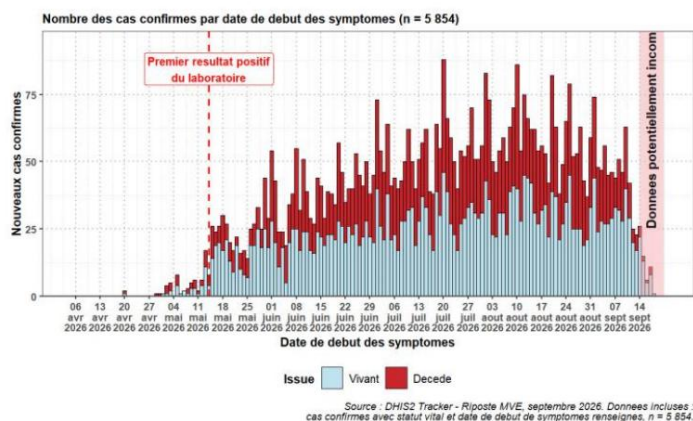
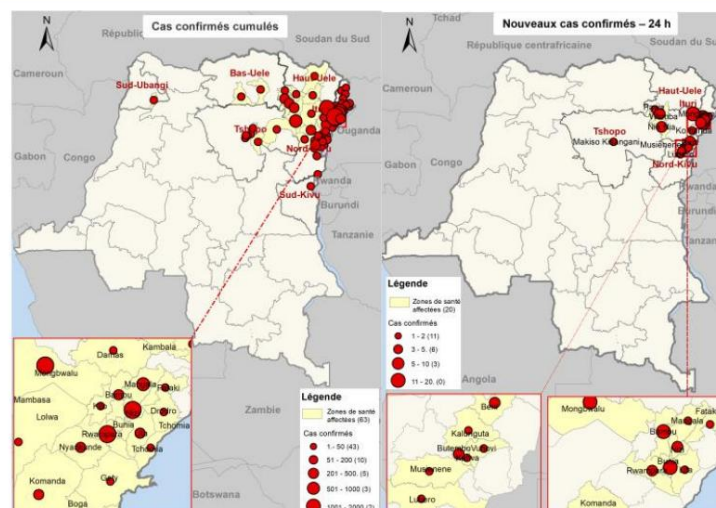
Tchomia	75	25	33.3%				0
To ventilate	N / A	493	N / A			6	6
North Kivu	1402	850	60.6%	14	4	3	7
Blessed	275	191	69.5%	4	2	3	5
Good	18	7	38.9%				0
Butembo	252	161	63.9%	3	0		0
Goma	1	0	0.0%				0
Kalunguta	43	15	34.9%	2	1		1
Katwa	542	343	63.3%	2	1		1
Kayna	1	0	0.0%				0
Kyondo	32	20	62.5%				0
Lubero	16	7	43.8%	1	0		0
Mabalako	24	11	45.8%				0
Manguredjipa	10	3	30.0%				0
Masereka	24	9	37.5%				0
Musienene	121	65	53.7%	1	0		0
Mutwanga	8	2	25.0%				0
Oicha	13	10	76.9%				0
Vuhovi	22	6	27.3%	1	0		0
Haut-Uélé	307	126	41.0%	5	0	0	0
Boma Mangbetu	36	15	41.7%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	98	42	42.9%				0
Pawa	68	32	47.1%	1	0		0
Rungu	2	1	50.0%				0
Wamba	99	34	34.3%	4	0		0
Tshopo	39	14	35.9%	1	0	1	1
Bafwasende	7	1	14.3%				0
Kabondo	5	1	20.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	20	9	45.0%	1	0	1	1
Mangobo	4	2	50.0%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas-Uélé	7	4	57.1%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	3	1	33.3%				0
Ganga	3	2	66.7%				0
South Ubangi	1	1	100.0%	0	0	0	0
Bulu	1	1	100.0%				0
Total	7672	3699	48.2%	58	13	10	23

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for September 19, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CTE/CT (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	8	7	15	5	7	3	0	10	4
Bas-Uélé	83	1	84	12	1	71	0	13	0
Ituri	985	90	1075	148	90	813	0	230	113
North Kivu	1079	38	1117	118	38	718	0	156	62
South Ubangi	ND	ND	0	ND	ND	ND	ND	ND	ND
Tshopo	108	0	108	8	0	100	0	8	0
Total General	2,263,136	2,399		291	136 1 705	0		417	179

Figure 1. Evolution of cases by date of symptom onset over the last 21 days**Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.**

1. COORDINATION

1.1. Main Actions

- Launch of vaccination of frontline providers in Bunia (Ituri province).

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

By the end of the day on September 19, **2,399 alerts** had been reported; **2,132 (88.9%)** were verified. Of these, **427 alerts were validated as suspected cases**, **417 (97.7%)** were investigated, and **179** were transferred to CT/CTE (Table 3).

Of the **32,958 contacts** being monitored, **26,319 were seen** in the last 24 hours, representing a follow-up rate of **79.9%**, lower than that observed the previous day (87.8%).

This proportion was 100.0% (42/42) in South Ubangi, 91.5% (193/211) in Bas-Uélé, 83.9% (271/323) in Tshopo, 81.4% (10,567/12,981) in North Kivu, 79.2% (13,748/17,365) in Ituri and 73.6% (1,498/2,036) in Haut-Uélé.

1.2. Update on checkpoint and point of entry (PoE/PoC) activities

1.2.1. Main actions

Three (3) alerts were notified to the PoE/PoC in the last 24 hours (2 in Ituri and 1 in North Kivu): two alerts concerning living persons and one intercepted body. Both (2) alerts concerning living persons were validated and investigated within 24 hours (100%; 2/2), and only one was transferred to the CT/CTE (50.0%; 1/2), while the body report was swabbed (100%).

Table 4. Status of key PoE/PoC indicators by province, as of September 19, 2026

Indicators	Ituri	North Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	93.3% (56/60) 100%	(20/20) 100% (18/18) 100%	(7/7) 96.2% (101/105)		
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%
Number of people who have passed PoE/PoC	137,484	96,239	18,664	13,207	265,594
% of travelers screened	95.0%	97.8%	99.7%	99.0%	96.6%
% of travelers who washed their hands	98.0%	97.8%	98.9%	99.0%	98.0%
% of travelers made aware	99.0%	97.8%	100.0%	100.0%	98.7%
Number of alerts notified	2	1	0	0	3
Number of validated live alerts (suspected cases)	1	1	0	0	2
Number of bodies recovered today	1	0	0	0	1
% of suspected cases transferred to CTE/CT	0% (0/1)	100% (1/1)	N / A	N / A	50.0% (1/2)
% of lifeless bodies swabbed/secured	100% (1/1)	N / A	N / A	N / A	100% (1/1)
% of suspected cases investigated within 24 hours	100% (1/1)	100% (1/1)	N / A	N / A	100% (2/2)

1.2.2. Challenges

Low alert encoding in DHIS2 Tracker and insufficient mobility in Rwampara and Bunia; persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri); In North Kivu, lack of support from security agents to the PoCs on the Butembo axis and threats against service providers.

1.3. Laboratory

1.3.1. Main actions

- **Ituri: 38 new positive results** (29 living and 9 deaths) out of 151 new samples received and analyzed (**positivity: 25.2%**).
- **North Kivu: 14 samples tested positive** (10 live and 4 dead) out of 116 samples analyzed (**positivity of 12.1%**).
- **Haut-Uélé: 5 new positive results** (5 live) out of 19 samples received and analyzed at the Isiro laboratory (**positivity: 26.3%**).
- **Tshopo: 1 new positive result (1 live sample) out of 4 samples received and tested (positivity: 25%)**.
- **Bas-Uélé: 2 samples received and analyzed (2 live).** Both samples were tested negative.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 23 rings were opened, representing an opening rate of 62.0%, and 42 households were decontaminated out of the 48 identified (87.5%); 5 health facilities were decontaminated and 52 households received hygiene kits in the Bunia health zone. Regarding blood and body surveys (BBS), 99 alerts were recorded (including 9 postponements from the previous day), 81 BBSs were carried out out of 97 (82.0%), and 67 bodies were swabbed out of 79; 12 could not be swabbed (15.2%).
- **In North Kivu**, risk assessment of staff exposure: 17/55 assessed are at high risk including: (3/18 in Musienene, 13/34 in Biena and 1/3 in Kyondo); Evaluation score card PCI WASH of CT MATANDA (84.9%), UCG (83%) and CTE Katwa with 93.7%; Monitoring of stock management of PCI/WASH inputs provided by UNICEF to the health zones of Butembo, Katwa and Kalunguta.
- **In Tshopo**, the third day of training for the 45 PPLs (Public Health Personnel) in the Kabondo Health Zone; decontamination of 21 rooms and 16 latrines at DHKis Hospital around the 29th, 32nd, 34th, 36th, and 38th confirmed cases, as well as 2 rooms and 2 latrines at the Joli Rêve Hotel and one room at CM Horizon; 13 ESS (Emergency Health Services) monitored and supported in 4 Health Zones, with 141 service providers briefed on standard precautions. As for the EDS (Emergency Health Services), an alert was issued. recorded.
- **In Bas-Uélé**, 2 rings were opened in the Viadana Health Zone, without cleaning or FOSA decontaminated due to a lack of WASH kits; 2 pre-triage units were installed at the Rubi Lisanga Health Center and the Mobenge Health Center, 26 service providers and 2 hygiene specialists were briefed in 6 ESS in the Buta and Ganga Health Zones, and the provision of IPC inputs with the support of FHI 360. As for the EDS, 1 alert was recorded in the Poko Health Zone and 1 body was swabbed.

1.4.2. Challenges

In North Kivu : the health zones of Biena, Masereka, Kyondo, Mabalako, and Vuhovi are not covered by decontamination activities due to a lack of training and motivation among the dedicated teams. **In Tshopo**, there is a shortage of infection prevention and control (IPC) supplies in social and health establishments (SHEs), households, schools, and markets; only partial coverage of SHEs with IPC kits (37.6%; 35/93); the incinerator at the Makiso-Kisangani General Referral Hospital remains out of service.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, 85 new admissions were recorded during the day, compared to 74 discharges, including 13 recoveries. Thus, 439 patients are hospitalized out of 1015 beds, representing an occupancy rate of 43.3%; 401 patients (91.3%) received hot meals, and children under LNPE/SLM were fully covered at the CTE (8/8), at the nursery (4/4), and in the community (30/30).
- **In North Kivu**, 54 new admissions were recorded compared to 63 discharges, including 2 recoveries; 376 patients are hospitalized, including 218 in standardized facilities with a capacity of 228 beds, representing an overall occupancy rate of 95.6%, close to saturation.
- **In Haut-Uélé**, 8 new admissions were recorded in the facilities, compared to 7 discharges (no recoveries). Thus, 60 patients are hospitalized out of 138 beds, representing an occupancy rate of 43.5%; 35 confirmed cases, 20 suspected cases, 73 accompanying persons, and 36 people with disabilities received hot meals, and the total number of recoveries stands at 102.
- **In Tshopo**, 4 new admissions were recorded against 2 discharges, including 1 non-case and 1 Deaths. Eleven (11) patients are in isolation out of 31 beds, representing an occupancy rate of 35.5%.

; 8 confirmed and 1 suspected received a hot meal and 42/42 (100%) frontline providers were served.

1.5.2. Challenges

Treatment remains partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases in Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 42% of hospitalized patients are treated outside of standardized facilities; no beds are available in Butembo and Katwa. **In Tshopo**, the lack of safe blood and limited technical resources persist. **In Bas-Uélé**, the absence of ETCs and ambulances hinders the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 3,576 people have been vaccinated to date out of 11,703 targeted TPL/PPL (30.6%), including 1,876 in Makiso-Kisangani, 486 in Kabondo, 422 in Mangobo, 326 in Tshopo, 291 in Bafwasende and 175 in Lubunga; the available stock is 500 frozen doses and 424 thawed doses expiring on September 24, 2026.
- **In Ituri**, organization of the official launch of the vaccination campaign for PPL at the Grand Laboratory of Bunia with the support of MSF.
- **In Bas-Uélé**, 987 PPLs have been vaccinated to date, including 550 in Buta, 324 in Ganga, 71 in Poko and 42 in Viadana; compassionate vaccination continues in Ganga and has started in Poko and Viadana, and the ultra-cold chain equipment has been received at the PEV antenna in Buta, with 87.9% of individual data encoded in DHIS2.

1.6.2. Challenges

Low adherence of PPL and TPL to vaccination in Tshopo, where coverage of the target remains at 30.6% and remains particularly low in Lubunga (11.0%), Mangobo (18.0%) and Tshopo (21.0%); risk of loss of 424 thawed doses expiring on September 24, 2026.

1.7. Continuity of Care

- **In Tshopo**, 146 cases received the experimental treatment, of which 19 were confirmed, and 127 are under post-exposure prophylaxis (92 completed, 11 interrupted and 24 ongoing); the partial evaluation of the Saint-Gaspard Polyclinic was carried out in the Makiso ZSU, none of the 29 PPLs of the structure being vaccinated to date, and the Mpox, cholera and measles drugs were delivered to the Tshopo and Mangobo ZS.
- **In Bas-Uélé**, the evaluation of the ESS in Makala and Triangle focused on vaccination coverage and the use of services; in Haut-Uélé, no continuity of care data was transmitted in the last 24 hours.

1.8. Risk Communication and Community Engagement (RCCE)

1.8.1. Main actions

- **In Ituri**, 29,805 home visits reached 94,472 people and 1,326 educational talks reached 18,236 people; 97 mass awareness campaigns reached 3,085 people, 36 community debates brought together 405 people and 5 advocacy sessions with 12 people; 398 community alerts were notified, including 62 deaths, 294 feedbacks were received, of which 245 were resolved (83.3%), and 12 community incidents, of which 9 were resolved (75.0%), for a completeness of 46.4% (13/28 health zones).
- **In North Kivu**, a message of commitment was delivered to the community by village leaders during a mass communication session organized by CREC in the village of Aloya in the Mabalako Health Zone; the aim is to encourage the entire community to support Ebola response efforts by adopting prevention measures and combating incidents
- **In Haut-Uélé**, one community dialogue reached 25 people (17 men and 8 women) and actions carried out in the 48 hours surrounding the latest confirmed cases reached 93

people in Pawa (19), Wamba (33) and Isiro (41); 6 suspected case alerts and 6 community death alerts were notified by community actors, 39 out of 160 feedbacks were resolved within 48 hours (24.4%) and 2 out of 98 incidents were resolved.

- **In Tshopo**, 3 mass awareness campaigns reached 2,077 people (1,206 men and 871 women) and 4 educational talks reached 165 people (74 men and 91 women); 3 interactive radio programs were hosted, 10 community alerts were notified and all resolved, and 2 feedback issues were resolved within 48 hours.

1.8.2. Challenges

Persistent community resistance: **in Tshopo**, the confirmed case of escape from the CTE (20th cases) remain at home, refusals to list contacts and investigate suspects are reported to DHK around the 29th case and 5 resistances have been notified of which only 2 were broken.

In Ituri, data reporting is low in some health zones (completeness: 46.4%; 13/28), there is a lack of quantified data on interventions around cases, and demotivation among stakeholders is linked to unpaid fees. **In North Kivu**, there is a refusal to register cases and carry out decontamination in Butembo and Biena, threats against an infection prevention and control (IPC) worker, and denial of the disease. **In Bas-Uélé**, there is a low reporting of community alerts, demotivation among community response teams, and impassable roads.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 21 of the last 46 confirmed cases (45.7%) and 21 of the 153 confirmed cases under follow-up (13.7%) benefited from SMSPS interventions, as well as 28 of the 81 new suspects (34.6%) and 111 of the 160 suspects under follow-up (69.4%); 222 caregivers and 82 PPLs were supported, 37 of the 43 recovered (86.0%) were followed up in the community and 56 results were announced, including 24 positive results; 10 of the 28 health zones are covered and 1 admission to a nursery was recorded, with 10 children being followed up there.
- **In North Kivu**, 20 people among confirmed cases in the last 48 hours and 218 former confirmed cases under follow-up received SMSPS interventions, as well as 217 new suspects and 276 suspects under follow-up; 97 results announced, including 20 positive cases, 88 households accompanied and 65 PPLs supported, 9/9 recovered and reintegrated and 125 men, 178 women and 69 children received psychoeducation sessions with the support of UNICEF and all 8 notified health zones are covered (100%).
- **In Tshopo**, 2 confirmed cases, 2 new suspected cases and 1 suspected case under follow-up benefited from SMSPS interventions; 6 contacts were supported at the CT of the Makiso-Kisangani HGR and 216 people benefited from psychoeducation, including 186 frontline providers and 30 community members from the Kabondo ZS.
- **In Haut-Uélé**, 4 new confirmed cases and 14 confirmed cases under follow-up benefited from SMSPS interventions, as well as 3 new suspects and 11 suspects under follow-up; 18 results were announced, including 4 positive ones, 54 caregivers and 44 PPLs were supported, 5 separated children were accompanied in the community and 3 EDS were supported.

1.9.2. Challenges

- **In Ituri**, low coverage of SMSPS interventions (10 of the 28 ZS covered) and need to strengthen actions around community deaths in Lita and Mongbwalu; in Tshopo, lack of care for an infant separated from its confirmed mother.

1.10. Logistics

1.10.1. Main actions

- Ensure the mobility of response teams in all affected provinces and continue construction work on the CTE at the Musienene General Referral Hospital in North Kivu.

1.10.2. Security

1.10.3. Main actions

- Continued security monitoring in the affected provinces and access zones difficult.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions

- **In Ituri**, a training-briefing on the PSEAH was organised in the Bunia Health Zone with the providers of the Ebola emergency coordination of the NGO ALIMA and the researchers of the INRB, bringing together 22 participants including 14 men and 8 women; awareness sessions were conducted in the Bunia and Rwampara Health Zones with the vendors of the central market, the car washers of the Ndimacho district, the staff and customers of the Ocean salon, the staff of IMC, the students and the members of CEMADAF.
- **At the national coordination level**, 221 staff members, including 141 men and 80 women, were briefed on the PSEAH (Public Health Support for Children) in the health zones of Bunia, Rwampara, Kisangani, and Butembo, and all signed the code of conduct; 189 people were sensitized in the community, including 124 women and 65 men, and three partners (WHO, Mercy Corps, and IMC) reported on their PSEAH activities. **To date**, 41 of the 119 monitored health zones (34.5%) are familiar with the reporting mechanism.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- In the last 24 hours, **58 new confirmed cases** have been reported, including **13 community deaths**. Additionally, **10 deaths within treatment centers** have been reported. The national total is therefore **7,672 confirmed cases, including 3,699 deaths (case fatality rate: 48.2%)**.
- Over the past 7 days, the provinces of Ituri, North Kivu, Haut-Uélé, Tshopo and Bas-Uélé have reported at least one confirmed case, highlighting active and persistent community transmission.
- Ituri remains the epicenter; it accounts for **77.1%** of confirmed cases and **65.5%** of new cases reported in the last 24 hours.
- The performance of epidemiological investigations remains satisfactory, with **97.7 %** of suspected cases investigated, although the contact tracing rate has declined (**79.9%**) due to lower completeness in several health zones.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to rapidly interrupt transmission chains: Rapidly increase reception capacities in ETC/CT in North Kivu and strengthen cross-border surveillance with South Sudan.
- Strengthen preparedness and response to Ebola Virus Disease-B in the provinces of the River Corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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